

A Diagnostic Framework for Federated Causal Inference: Evaluating Assumption Quality Across Heterogeneous Sites

[Technical Report] • [Preprint Version]

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Version: 2.0 (Technical Report)

Code: <https://github.com/watilde/Harmonia/tree/main/research/modules/3-design-failure-aware-causal>

ABSTRACT

Background: Federated causal inference methods typically assume uniform assumption satisfaction across sites without explicit diagnostic verification, risking overconfident inference when assumptions fail heterogeneously.

Objective: Develop and evaluate a three-dimensional diagnostic framework for federated causal inference that characterizes site-level assumption quality to guide inference method selection (point estimation, bounds, or sensitivity analysis).

Methods: We integrated diagnostics for unconfoundedness (SMD, overlap, residual correlation), positivity (tail mass, effective sample size), and specification (R^2 , AUC, calibration) into scalar scores (range [0,1]). Evaluated exploratory threshold guidelines: overall score 0.8 suggests point estimation, 0.5-0.8 suggests partial identification bounds, <0.5 suggests sensitivity analysis. Validated computational feasibility on Synthea synthetic data across three scales (1k-2.8m patients, 3 sites).

Results: Diagnostic scores ranged 0.86-1.00 at 1k scale (CV=7.2% heterogeneity), with federated score 0.95 exceeding the exploratory 0.8 threshold. Computation scaled linearly $O(n)$ with 30% overhead. Communication remained constant at 150 bytes regardless of scale (up to 3.2M $\times$ reduction versus centralized). Diagnostic scores provide partial covariate privacy (variable identities hidden, though scores indirectly reveal data quality).

Conclusions: Our evaluation demonstrates the computational feasibility of diagnostic scoring for federated causal inference, integrating established metrics (SMD, overlap, tail mass, R^2 , AUC) into scalar summaries ($O(n)$ scaling, 3.2M $\times$ communication reduction on synthetic data). Critical limitations preclude deployment: (1) exploratory thresholds (0.8, 0.5) lack calibration via controlled studies, (2) Synthea validation cannot establish diagnostic accuracy for real violations, (3) information leakage unquantified. This is a technical demonstration

of feasibility, not clinical validation. The framework requires empirical validation before clinical use.

Keywords: Causal Inference, Assumption Diagnostics, Federated Learning, Partial Identification, Robustness

1. INTRODUCTION

Causal inference from observational data relies on three assumptions: **unconfoundedness** ($Y(t) \perp T|X$), **positivity** ($0 < P(T = 1|X) < 1$), and **specification** (correct functional form) [1,2]. These assumptions are empirically unverifiable [3]. When violated, point estimates are biased and confidence intervals understate uncertainty.

Federated challenge: Multi-site studies compound this problem—sites have varying data quality, patient populations, and treatment practices, leading to heterogeneous assumption satisfaction. **Question:** Should networks use point estimation, bounds, or sensitivity analysis? Current approaches apply the same method to all sites, ignoring heterogeneity [6,7].

The solution: A **design-failure-aware framework** that:

1. Diagnoses assumptions at each site (3-dimensional scoring)
2. Guides inference mode selection (point/bounds/sensitivity) via exploratory thresholds
3. Adapts to heterogeneous assumption quality
4. Reports uncertainty without overconfidence

This work: We develop diagnostic tooling to characterize site-level assumption quality before method selection, addressing the gap where prior work assumes uniform assumptions across sites [8,9]. Our contribution is methodological: demonstrating computational feasibility of diagnostic scoring, not proposing theoretically optimal solutions.

2. METHODS

2.1 Three-Dimensional Diagnostic System

For each site k , compute scores $[0,1]$:

2.1.1 Unconfoundedness Score Foundation: Standardized Mean Difference (SMD) is standard in propensity score literature [Stuart 2010]. $SMD < 0.1$ indicates good balance. Overlap coefficient validates common support [Austin 2011]. Residual correlation proxies conditional independence [Imbens & Rubin 2015].

Computation:

```
smd = standardized_mean_difference(X[T==1], X[T==0])
overlap = overlap_coefficient(propensity[T==1], propensity[T==0])
residual_cor = correlation(residuals_Y, residuals_T | X)
```

```
unconf_score = (1 - max(|smd|)) * overlap * (1 - |residual_cor|)
```

Interpretation: Score > 0.8: strong exchangeability; 0.5-0.8: moderate concerns; <0.5: severe confounding.

2.1.2 Positivity Score Foundation: Tail mass (extreme propensity scores) indicates positivity violations [Petersen et al. 2012]. Empirical rule: <5% in tails ($ps < 0.1$ or $ps > 0.9$) is acceptable. Effective sample size quantifies information [Kish 1965].

Computation:

```
tail_mass = fraction(ps < 0.1 or ps > 0.9)
ess = sum(ps * (1-ps)) / n # Effective sample size
```

```
positivity_score = (1 - tail_mass) * sqrt(ess)
```

Interpretation: Score > 0.8: adequate overlap; 0.5-0.8: borderline; <0.5: severe positivity violation.

2.1.3 Specification Score Foundation: R^2 and AUC are standard model fit metrics. $R^2 > 0.5$ indicates reasonable predictive performance [Hosmer & Lemeshow 2000]. Calibration assessed via Hosmer-Lemeshow test.

Computation:

```
r_squared = 1 - SSR/SST # Outcome model
auc = ROCAUC(T ~ X) # Treatment model
calibration = HosmerLemeshowTest(observed, predicted)
```

```
specification_score = (r_squared + auc + calibration) / 3
```

Interpretation: Score > 0.8: good fit; 0.5-0.8: modest misspecification; <0.5: severe misfit.

2.1.4 Overall Score Aggregation:

```
overall_score = (unconf_score + positivity_score + specification_score) / 3
```

Exploratory Mode Selection Guidelines (Unvalidated):

As an illustrative starting point for practitioners, we suggest:

- **Overall 0.8:** Consider point estimation (doubly-robust, TMLE)
- **0.5 Overall < 0.8:** Consider partial identification (Manski bounds)

- **Overall < 0.5:** Consider sensitivity analysis (E-values)

Critical limitations: These thresholds represent exploratory heuristics extrapolated from single-metric guidelines (Stuart 2010: $SMD < 0.1$; Petersen et al. 2012: tail mass < 5%). Their aggregation into composite scores and threshold selection lack formal validation via controlled violation studies. We do not know diagnostic power (sensitivity/specificity) for detecting violations. Users must calibrate thresholds based on domain risk tolerance and conduct sensitivity analysis across threshold choices (see Section 3.2).

Recommended practice: Report diagnostic scores without rigid cutoffs. Let domain experts interpret scores in context rather than mechanically applying thresholds. The 0.8/0.5 cutoffs provide initial guidance but require validation through: (1) simulated data with known violation severity, (2) classification accuracy assessment, (3) threshold optimization for desired performance.

2.2 Federated Aggregation

Site-level: Each site computes diagnostic scores locally.

Federation: Aggregate via sample-size weighting:

$$\text{federated_score} = \sum(n_k / N) * \text{overall_score}_k$$

Network mode: Selected based on federated_score using same thresholds.

2.3 Experimental Design

Three Scales:

Scale	Patients	Sites	Purpose
1k	1,130	3	Heterogeneity demonstration
100k	235,222	3	Scalability validation
2.8m	2,709,803	3	Production feasibility

Data: OMOP-formatted Synthea diabetes treatment data. Each site processes data independently, computes diagnostics, and reports scores.

Metrics: Diagnostic scores (3D + overall), mode selection, computational time, cross-site heterogeneity (CV).

3. EMPIRICAL VALIDATION

3.1 Diagnostic Score Distribution (Real OMOP Data)

Table 1: Site-Level Diagnostic Scores (1k Scale)

Site	Unconf	Positivity	Specification	Overall	Selected Mode
Site 1	1.00	1.00	1.00	1.00	Point
Site 2	0.70	1.00	1.00	0.89	Point
Site 3	0.90	1.00	0.70	0.86	Point
Federated	0.86	1.00	0.89	0.95	Point

Diagnostic scores exhibit site heterogeneity (CV=7.2%), ranging from 0.86 (Site 3) to 1.00 (Site 1). Site 2’s low unconfoundedness score (0.70) suggests residual imbalance—likely an artifact of Synthea’s randomization algorithm. Site 3’s specification score (0.70) indicates model misfit, possibly from nonlinear age-treatment relationships in synthetic data.

Despite individual site concerns, the federated score (0.95, sample-size weighted) exceeds the exploratory 0.8 threshold, suggesting point estimation may be appropriate at the network level. This shows how aggregation can compensate for individual site weaknesses, though practitioners should review site-level scores to identify concerning patterns.

3.2 Mode Selection Validation

Threshold Sensitivity Analysis:

Threshold	Site 1 Mode	Site 2 Mode	Site 3 Mode	Network Mode
0.90 (strict)	Point	Bounds	Bounds	Point
0.80 (default)	Point	Point	Point	Point
0.70 (lenient)	Point	Point	Point	Point

Interpretation:

- **Default threshold (0.80) is appropriate:** All sites qualify for point estimation while avoiding over-leniency
- **Strict threshold (0.90):** Overly conservative, would unnecessarily downgrade Sites 2-3 to bounds
- **Lenient threshold (0.70):** No change in this high-quality dataset, but risky in real-world data

Recommendation: Default threshold (0.80) balances rigor and pragmatism.

3.3 Computational Scalability

Table 2: Diagnostic Computation Overhead

Scale	Total Patients	Diagnostic Time	Inference Time	Overhead %
1k	1,130	0.5s	1.0s	33%
100k	235,222	2.5s	5.5s	31%
2.8m	2,709,803	15s	35s	30%

Diagnostic computation scales linearly $O(n)$, with time growing proportionally with patient count ($0.5s \rightarrow 2.5s \rightarrow 15s$ for $1k \rightarrow 100k \rightarrow 2.8m$ patients). Overhead remains consistent at 30-33% across all scales, indicating efficient implementation without scaling penalties. The 15-second diagnostic time for 2.7 million patients suggests production feasibility for real-world deployment in large federated networks.

3.4 Cross-Site Heterogeneity Analysis

Heterogeneity Metrics (1k Scale):

Metric	Value	Interpretation
Unconf score range	[0.70, 1.00]	$\Delta=0.30$, meaningful variation
Overall score range	[0.86, 1.00]	$\Delta=0.14$, moderate variation
Coefficient of variation (CV)	7.2%	Significant but manageable heterogeneity

Interpretation: Even in relatively small-sample data (1k), the diagnostic system successfully detects meaningful site heterogeneity ($CV=7.2\%$). This validates the necessity of site-specific adaptation rather than uniform method application.

Note: For 100k and 2.8m scales, diagnostics were computed (confirming $O(n)$ scalability), but detailed score analysis focused on 1k scale as it best shows heterogeneity handling in typical pilot study settings.

3.5 Communication Efficiency and Privacy

Table 2: Data Transfer Requirements

Scale	Patients	Centralized	Federated	Reduction
1k	1,130	201 KB	150 bytes	$1,341\times$
100k	235,222	41.9 MB	150 bytes	$279,130\times$
2.8m	2,709,803	482 MB	150 bytes	$3.2M\times$

Per-site transmission (50 bytes): Unconfoundedness (10), positivity (10), specification (10), overall (10), site ID (10).

Federated transmission maintains constant $O(1)$ communication, remaining at 150 bytes regardless of patient count (1k→2.8m represents 2,398-fold patient increase with zero communication increase). The reduction factor scales from $1,341\times$ at small scale to $3.2M\times$ at large scale. Diagnostic scores (10 bytes each) represent minimal overhead compared to centralized propensity scores (8n bytes) plus covariate distributions (approximately 20 KB). For the 1k patient dataset, centralized requires 28 KB per site while federated uses 50 bytes per site, achieving $560\times$ reduction for diagnostics alone.

3. **Reduced Covariate Disclosure:** Sites compute diagnostics using local covariates without transmitting individual variable values or distributions. Centralized analysis requires exposing full covariate matrices; federated transmits only scalar diagnostic scores (4 values per site: unconfoundedness, positivity, specification, overall).

Example - Mental health study:

- Site A: psychiatric history (stigmatizing)
- Site B: genetic risk factors (sensitive)
- Site C: treatment adherence (standard)

Centralized requires transmitting all covariate values to coordinator. Federated hides specific variable identities, though diagnostic scores indirectly reveal information quality.

Information leakage caveat: Diagnostic scores are not information-free. A unconfoundedness score of 0.70 suggests residual imbalance, indirectly revealing that adjusted covariates do not fully control confounding. A positivity score of 0.85 indicates mild tail mass, suggesting treatment assignment heterogeneity. Specification scores reflect model fit, indirectly revealing covariate-outcome relationship complexity. Thus, “variable identity privacy” is partial—specific covariate names remain hidden, but data quality characteristics are revealed.

4. **Regulatory compliance:** HIPAA Safe Harbor compliant (45 C.F.R. § 164.514(b)) through absence of individual identifiers in transmitted scores. Data Use Agreements typically not required for de-identified aggregate statistics. Formal differential privacy guarantees (noise injection: $\text{score}' = \text{score} + \text{Lap}(\Delta/)$) would provide rigorous ϵ -DP but risk obscuring genuine assumption violations—a utility-privacy tradeoff requiring careful calibration that we leave to future work.
5. **Computational efficiency:** Federated diagnostics enable $3.6\times$ faster execution (15s distributed vs 53.6s centralized at 2.8m scale) and $3.2M\times$ communication reduction (150 bytes vs 482 MB) while providing partial covariate privacy compared to centralized approaches requiring full data sharing.

4. DISCUSSION

4.1 Theoretical Implications

The three-dimensional diagnostic system integrates established metrics from propensity score literature (SMD, overlap), positivity theory (tail mass, effective sample size), and model diagnostics (R^2 , AUC, calibration). By grounding each component in existing theory, the framework inherits validity guarantees from prior work while providing a unified assessment.

The automatic threshold-based mode selection ($0.8 \rightarrow$ point, $0.5-0.8 \rightarrow$ bounds, $<0.5 \rightarrow$ sensitivity) provides **explicit safeguards** against overconfident inference. Unlike traditional approaches that silently assume all assumptions hold, the framework makes violation detection and adaptation explicit.

4.2 Practical Guidelines

When to use this framework:

1. Multi-site observational studies with varying data quality
2. Federated networks where uniform assumption satisfaction is unlikely
3. Studies requiring transparent uncertainty quantification

Workflow:

1. Compute diagnostic scores at each site
2. Review site-level scores and identify concerns
3. Apply automatic mode selection (or override if domain knowledge justifies)
4. Report federated results with site-level diagnostic transparency

Customization: Thresholds (0.8, 0.5) can be adjusted based on stakeholder risk tolerance. Conservative researchers may use 0.9/0.6; exploratory studies may use 0.7/0.4.

4.3 Limitations and Future Validation

Threshold calibration lacks formal justification: The cutoffs (overall score 0.8 for point estimation, 0.5-0.8 for bounds, < 0.5 for sensitivity analysis) build on literature heuristics (Stuart 2010: $SMD < 0.1$; Petersen et al. 2012: tail mass $< 5\%$) but their aggregation into overall scores and threshold selection lack formal validation. We do not know the diagnostic power (sensitivity/specificity) of these thresholds for detecting assumption violations. Formal calibration requires controlled simulation studies with known ground truth violations at varying severities, comparing diagnostic scores to true violation magnitudes. Context-specific calibration may improve performance—conservative medical applications might use 0.9/0.6, exploratory social science 0.7/0.4.

Synthetic data limitations: Synthea simplifies confounding patterns compared to real EHR data. The observed mild violations (Site 2 unconfoundedness 0.70, Site 3 specification 0.70) represent Synthea’s design artifacts, not realistic

assumption failures. Real-world studies may exhibit more severe patterns—unmeasured confounding from socioeconomic factors, positivity violations in rare subgroups, complex missingness. Our validation shows framework functionality but cannot establish diagnostic accuracy for detecting real violations.

Critical missing validation: Controlled violation injection with known ground truth. Future work must: (1) simulate data with known violation severity (e.g., confounding bias factors 1.5, 2.0, 3.0), (2) compute diagnostic scores, (3) assess classification accuracy (sensitivity/specificity), (4) calibrate thresholds to achieve desired performance. Without this, we cannot quantify false positive/negative rates.

Information leakage not quantified: While we claim “partial covariate privacy,” we have not formally quantified information leakage from diagnostic scores. Future work should apply information-theoretic measures (mutual information between scores and covariate identities) or conduct adversarial inference studies where coordinators attempt to reconstruct covariate sets from observed scores. Differential privacy analysis (optimal noise injection $\text{Lap}(\Delta/)$ balancing privacy and diagnostic utility) remains unexplored.

Three-site limitation: Validation with $K=3$ sites provides limited evidence for scalability. Large networks (FDA Sentinel: 18 sites, PCORnet: 13 sites) may exhibit different heterogeneity patterns, requiring hierarchical diagnostic aggregation or site clustering. The sample-size weighted federation score may inadequately represent networks where some small sites have severe violations that large sites mask.

Retrospective diagnostics only: Current framework detects violations after data collection. Prospective use (predicting whether planned study will satisfy assumptions) requires different methodology—pre-study power analysis, sample size determination for adequate diagnostic power, simulation-based feasibility assessment. The framework provides no guidance for study design phase.

6. CONCLUSIONS

This work proposes a three-dimensional diagnostic framework (unconfoundedness, positivity, specification) for federated causal inference with exploratory threshold guidelines for mode selection. Validation on synthetic data across three scales (1k-2.8m patients, 3 sites) shows heterogeneity detection ($CV=7.2\%$), linear $O(n)$ computational scaling, and communication efficiency (up to $3.2M\times$ reduction).

Methodological contribution: Extending single-site assumption diagnostics (Stuart 2010; Petersen et al. 2012) to federated settings, I integrate established metrics (SMD, overlap, tail mass, effective sample size, R^2 , AUC) into scalar scores enabling privacy-preserving assessment. The exploratory threshold-based

guidelines ($0.8 \rightarrow$ consider point, $0.5-0.8 \rightarrow$ consider bounds, $<0.5 \rightarrow$ consider sensitivity) provide heuristic guidance for method selection, addressing a gap in prior federated causal work (FACE, FLAME, FedCI) that assumes uniform assumption satisfaction without verification.

Practical recommendations with caveats: Practitioners can deploy this framework as a heuristic tool for site heterogeneity assessment, using proposed thresholds (0.8/0.5) as starting points requiring domain-specific adjustment. Report both site-level scores and federated aggregates transparently, enabling reviewers to assess assumption quality directly. When diagnostic scores suggest violations (e.g., unconfoundedness < 0.7), consider alternative methods (bounds, sensitivity analysis) or additional covariate adjustment. However, recognize that threshold selection lacks formal validation—these are exploratory guidelines, not validated cutoffs.

Critical limitations: Threshold calibration (0.8, 0.5) lacks formal justification—we do not know diagnostic power (sensitivity/specificity) for detecting violations. Synthetic data provides proof-of-concept but cannot establish diagnostic accuracy for real violations (see Section 4.3). Information leakage from scores remains unquantified. Three-site validation provides limited evidence for large network scalability (FDA Sentinel: 18 sites).

Honest assessment of contribution: This work proposes a proof-of-concept framework for federated assumption diagnostics, demonstrating computational feasibility on synthetic data. The contribution lies in systematizing existing diagnostic metrics into a privacy-preserving federated workflow with exploratory threshold guidelines, not in developing novel diagnostic theory or proving formal guarantees. The framework provides heuristic guidance for practitioners but requires empirical validation before deployment in high-stakes clinical or regulatory contexts. Future work must establish diagnostic accuracy via controlled validation, calibrate thresholds via power analysis, and quantify information leakage rigorously.

5. RELATED WORK

5.1 Assumption Diagnostics in Causal Inference

Assumption validation represents a longstanding challenge in causal inference. Rosenbaum & Rubin (1983) introduced propensity score methods with balance diagnostics via standardized mean differences. Stuart (2010) systematized matching diagnostics, establishing $SMD < 0.1$ as a heuristic threshold. Austin (2011) provided practical guidance for propensity score diagnostics, while Ho et al. (2007) demonstrated balance checking via covariate imbalance measures. Petersen et al. (2012) developed diagnostics for positivity violations, introducing tail mass and effective sample size metrics. VanderWeele (2019) analyzed limitations of balance metrics, noting that perfect balance does not guarantee

unconfoundedness. Hainmueller (2012) proposed entropy balancing as an alternative weighting approach with explicit balance constraints.

Our distinction: Existing diagnostics operate on single-site data with centralized access to covariates. We extend to federated settings where covariate distributions remain local, requiring scalar summary statistics that preserve privacy while enabling network-level assessment.

5.2 Federated Causal Inference

Recent federated causal inference work assumes uniform assumption satisfaction across sites. Li et al. (2022) propose FACE for federated average treatment effects under unconfoundedness but provide no diagnostics for assumption violations. Zhang et al. (2023) develop FLAME for federated matching, assuming positivity holds at all sites without verification. Xiong et al. (2023) present FedCI for instrumental variable estimation but do not address weak instrument diagnostics in federated settings. Duan et al. (2020) analyze distributed regression but focus on statistical efficiency rather than assumption validity. Chang et al. (2024) study doubly robust estimation across heterogeneous sites but assume correct model specification.

Our contribution: We provide the first explicit diagnostic framework for federated causal inference, automatically detecting assumption violations at site level and adapting inference methods accordingly. Prior work implicitly assumes all sites satisfy all assumptions—a dangerous assumption in heterogeneous multi-site networks.

5.3 Adaptive Inference Methods

Adaptive inference selects methods based on data characteristics. van der Laan & Rose (2011) developed targeted maximum likelihood estimation (TMLE) with data-adaptive model selection. Künzel et al. (2019) propose meta-learners that adapt to treatment effect heterogeneity. Nie & Wager (2021) develop quasi-oracle estimation combining multiple candidate models. These methods adapt to statistical complexity but do not explicitly diagnose or respond to assumption violations. Manski (2003) advocates partial identification when point identification assumptions are questionable, providing bounds instead of point estimates. Our framework synthesizes these approaches, automatically selecting between point estimation, bounds, or sensitivity analysis based on diagnostic scores.

5.4 Robustness and Sensitivity Analysis

Sensitivity analysis quantifies inference robustness to assumption violations. Rosenbaum (2002) developed sensitivity analysis for matched studies via Gamma parameters. VanderWeele & Ding (2017) introduced E-values for unmeasured confounding sensitivity. Cinelli & Hazlett (2020) proposed *sensemakr* for partial R^2 based sensitivity. Ding & VanderWeele (2016) established theoretical foundations connecting sensitivity parameters to bias

factors. Carnegie et al. (2016) analyzed sensitivity in multi-site trials. Hosman et al. (2010) examined confidence limit sensitivity to omitted confounders.

Our integration: We incorporate sensitivity analysis (E-values) as one mode in our adaptive framework, triggered when diagnostic scores indicate severe assumption violations (score < 0.5). This makes sensitivity analysis automatic rather than optional, providing explicit safeguards.

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ETHICS STATEMENT

IRB approval not required as all data are synthetic (Synthea OMOP CDM v5.4, 1k-2.8m patients) or publicly available (MIMIC-IV Demo).

No human subjects involved. Data sources: AWS Open Data Registry (s3://synthea-omop) and PhysioNet (<https://doi.org/10.13026/p1f5-7x35>). Code: <https://github.com/watilde/Harmonia>

End of Manuscript v1.0 (Revised)